

MY PERSONAL ASTHMA ACTION PLAN

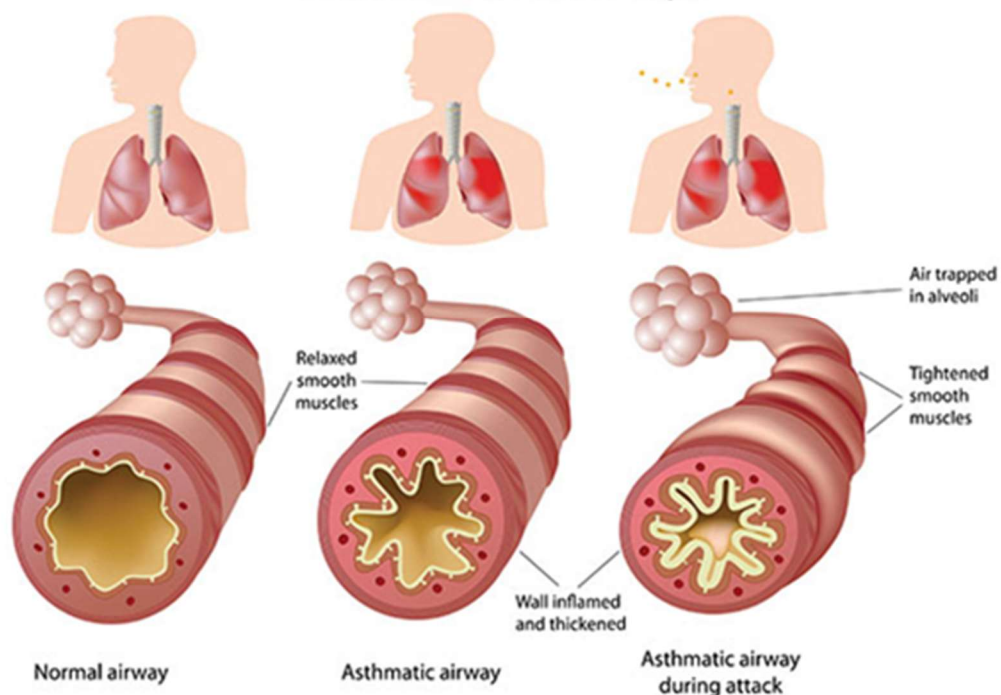
Name:	Given Name Surname	Height	Single Code Entry: Standing height	Weight	Single Code Entry: Body weight
DOB:	Date of Birth	Best Peak Flow		Predicated Peak Flow:	
NHS No:	NHS Number	Allergies:		Triggers:	
Hospital Number:	Hospital Number				
GP Name:	Registered GP Full Name			Tel:	Organisation Telephone Number
Practice Nurse Name:				Tel:	Organisation Telephone Number
Hospital Practitioner:				Tel:	

MY ASTHMA TREATMENT

Medication / Inhaler Colour	Strength	Frequency (inhalation/times per day)	Device / Type of Medication
Issued by:		Date:	Short date letter merged

The Four Stages of Asthma Control	Key messages:	Recommended Actions
Stage 1. Your asthma is under good control when: - There is no cough, wheeze, chest tightness or shortness of breath. - You are able to do usual activities. - Your peak flow is 75% - 100% of best (5 years and above). 100% 75%	Peak Flow recorded from 5 years and above.	Preventer medications: An asthma review must be attended once or twice per year. If asthma is well controlled, the health care professional may trial a step down of your regular treatment.
	Remember: Continue to take preventer medication as directed.	
Stage 2. Your risk of an asthma attack is increased with any of the following: - Cough, wheeze, chest tightness or shortness of breath. - Waking at night due to asthma symptoms, including a cough. - You are able to do some, but not all usual activities. - Your peak flow is 60%-75% of best (5 years and above). 75% 60%	Important: Seek a medical review if reliever medication is: Needed three times a week or more. Needed regularly for more than 24 hours. Not lasting at least 4 hours.	Option A- For reliever (blue) inhaler Take TWO puffs via MDI and spacer device (one puff every 30-60 seconds). If no relief after 5-10 minutes take another TWO puffs (one puff every 30-60 seconds) up to a maximum of FOUR puffs . If still no relief move to stage 3. -OR- Option B- Maintenance And Reliever Therapy (MART) Take ONE puff/inhalation of . If symptoms are not improving after 5 minutes take ONE more puff/inhalation (if still no relief move to stage 3). Prednisolone may be required at this stage following medical review.
Stage 3. You are having an asthma attack when: - Symptoms are worsening. - Reliever is less effective and is needed more often than usual. - Reliever is not lasting 4 hours. - You are breathless on normal activity. - Your peak flow is 60% or less (5 years and above). 60%	Important: Seek immediate medical review if reliever medication is: Needed regularly for more than 24 hours. Not lasting at least 4 hours. You are still symptomatic after maximum use of reliever medication.	Option A- For reliever (blue) inhaler Take TWO puffs via MDI and spacer device (one puff every 30-60 seconds). If no relief after 5-10 minutes take another 2 puffs (one puff every 30-60 seconds). If no relief after 5-10 minutes take 2 more puffs (one puff every 30-60 seconds). You have now had 6 puffs . If still no relief after 6 puffs move to stage 4. -OR- Option B- Maintenance And Reliever Therapy (MART) Take ONE more puff/inhalation of . If symptoms are not improving after 5 minutes take ONE more puff/inhalation. No more than SIX puffs/inhalations on any single occasion maximum of puffs/inhalations daily (if still no relief move to stage 4). You may need to start a course of Prednisolone mg once daily for days or until symptoms resolve.
Stage 4. You are having a severe asthma attack when: - Symptoms continue to get worse. - Symptoms are not responding to reliever. - You are too breathless or exhausted to speak. - Peak flow is 40% or less (5 years and above). 40%	Important: Arrange for an urgent medical review. -or- Call 999 immediately if there is no or poor response to 10 puffs.	- Call for help. - Take one puff of reliever (blue) inhaler via MDI and spacer device (one puff every 30-60 seconds) up to a total of 10 puffs. - Wait 5 –10 minutes: if symptoms have improved still arrange an urgent medical review with a GP or attend the Emergency Department. - If symptoms are not improving repeat step 2 and immediately call 999 for an ambulance. - If symptoms are no better after repeating step 2 and the ambulance has still not arrived contact 999 again immediately.

Asthma and Your Airways



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Handy Tips

Do not run out of your inhalers!

- Each inhaler comes with a set amount of doses. Please count out the doses so you do not run out of treatment.
- Once empty it will only deliver the propellant.
- Always carry your reliever inhaler/MDI and spacer with you including at school and work.

Inhaler maintenance

- Rinse your mouth and gargle after using steroid Inhalers.
- Regularly check the expiry dates of **all** your medicines.
- Wash the spacer once weekly in warm soapy water and allow to air dry.
- If you use a spacer, replace this every 6 to 12 months.
- **If your reliever medication is used regularly three times a week or more, please arrange a medical review.**

Asthma Review

- Please bring all of your medications and personal asthma action plan with you if you are admitted to hospital or if you attend any outpatient appointments.
- Please arrange a review with your GP, or asthma nurse within two working days after an admission to hospital or after receiving emergency care.
- If you have commenced a rescue course of prednisolone, you should contact your GP for a review.
- **You will need to attend your asthma reviews and take your medication as directed.**